

BFHL HealthPay-BajajPay QR Transition / Merger

Design Thinking-Led Product Case Study

Parallel HealthPay Initiative | Requirement Definition, Solutioning, Migration Planning, UAT Support

Positioning Note

This was a parallel HealthPay initiative during the Partner Center period. Samadhan contributed to requirement definition, solutioning, integration coordination, QR/merchant migration planning, SOP/logistics/service design, and UAT support. He was moved to another project during final stages, so this document avoids claiming full final rollout ownership.

Dimension	Confirmed / Source-Backed Detail
Project Name	HealthPay-BajajPay QR Transition / Merger
Organization Context	BFHL / HealthPay with BFL BajajPay integration
Timeline	Parallel to Partner Center period; source documents indicate 2023 delivery and migration planning milestones.
Project Type	Payment infrastructure migration, QR transition, onboarding and integration design
Primary Scope	Provider onboarding, QR generation/mapping/logistics, transaction feeds, settlement feeds, refunds, support operations, and health plan consumption flows.
Your Role	Product requirement definition, solutioning, migration planning, SOP/logistics/service design, UAT support, and coordination until transition to another project.
Final Stage Caveat	Moved to another project during final stages; final commercial rollout ownership should not be overclaimed.

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1. Executive Summary

The HealthPay-BajajPay QR Transition / Merger was a parallel HealthPay initiative during the Partner Center period. The project focused on migrating BFHL HealthPay provider QR infrastructure to BajajPay while keeping HealthPay, BajajPay, BFHL provider systems, customer journeys, transaction feeds, settlement feeds, refund workflows, communication, help/support, and QR logistics aligned.

The initiative was not merely a QR replacement exercise. It required coordinated product design across merchant onboarding, existing provider migration, QR generation, QR mapping, transaction status APIs/webhooks, settlement APIs/SFTP feeds, refund callbacks, health plan consumption on 3-in-1 journeys, merchant capabilities, and joint customer/merchant support models.

Samadhan worked on the project in parallel to Partner Center and contributed to requirement definition, solutioning, integration coordination, migration planning, QR logistics/service SOPs, merchant support flows, and UAT support. The source note states that integration between BajajPay and HealthPay was completed, UAT was successful, and code was moved to production from both ends. However, CUG and GTM rollout were still dependent on FOS onboarding and later migration activities. Samadhan was moved to another project during final stages, so this document positions his contribution honestly around solutioning and transition support rather than final rollout ownership.

Core Project Thesis

The real problem was not "move HealthPay QR to BajajPay QR". The real problem was to migrate payment infrastructure without breaking provider onboarding, transaction visibility, settlement continuity, QR logistics, health plan consumption, communication ownership, support operations, and compliance migration.

2. Project Context

Area	Detail
Existing System	HealthPay QR/payment infrastructure used by BFHL providers.
Target Infrastructure	BajajPay QR infrastructure powering QR generation and payment rails.
Provider Scope	Doctors, labs, and hospitals.
Migration Scope	Existing HealthPay providers to BajajPay QR; new BFHL merchants to be onboarded on BajajPay platform.
Systems Involved	BFHL systems, BFL/BajajPay systems, S1 app, SFDC, DRx/PMS, Provider Portal/Partner Center, HRx/HealthRx app, 3-in-1 app, payment dashboards, settlement feeds, APIs and webhooks.
Business Dependency	Provider migration, FOS onboarding on S1, QR logistics, transaction/settlement continuity, support ownership, and GTM sequencing.
Final Stage Status	Integration and UAT completed; code moved to production; CUG and migration rollout depended on onboarding and GTM execution.

3. Source-Backed Evidence

Source Artifact	Evidence Used in This Case Study
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BajajPay HealthPay Note.docx	Integration completion, successful UAT, production movement from both ends, CUG dependency on FOS onboarding, GTM plan, migration targets, metrics, challenges, next steps, and project tracker ownership areas.
BajajPay HealthPay Requirement Doc from BFHL v3.docx	Project objective, merchant onboarding, QR generation, QR logistics, QR scanning, multi-QR, transaction status API/webhook, settlement API, refund API/webhook, merchant capabilities, help/support, and case-management requirements.
BajajPay HealthPay merger decision.pptx	Builds needed, integration areas, logistics, communication ownership, support model, and challenge/risk framing.
S1 onboarding / transition trackers	Operational dependency around FOS onboarding and merchant hierarchy mapping.
User confirmation	Samadhan worked on this in parallel to Partner Center and moved to another project during final stages.

4. Strategic Framing

A weak framing would be: Worked on BajajPay QR migration. That sounds like an operational QR replacement project.

The stronger framing is: Contributed to the HealthPay-BajajPay QR transition initiative, defining requirements and solutioning for provider onboarding, QR migration, transaction and settlement feeds, refund workflows, QR logistics, merchant support, health-plan consumption, and cross-system synchronization across BFHL and BFL ecosystems.

Incorrect / Weak Positioning	Correct / Strong Positioning
QR replacement project	Payment infrastructure transition and partner migration program
Moved old QR to new QR	Mapped HealthPay UPI IDs to BajajPay UPI IDs while preserving transaction and settlement continuity
Technical integration only	Cross-company product, operations, logistics, support, and compliance initiative
Final rollout ownership	Requirement definition, solutioning, integration coordination, UAT support, and transition before final rollout

5. Core Design Thinking Insight

The key design-thinking insight was that payment migration is not just a technology migration. It changes the operating model for merchants, field teams, customers, finance, support, logistics, and provider-facing systems.

If the QR migration was handled only as a payment-rail switch, it could break transaction visibility, settlement tracking, refund handling, merchant support, health-plan usage, provider communications, and field logistics. Therefore, the product problem had to be reframed as an end-to-end transition design problem.

Reframed Problem

From: Replace HealthPay QR with BajajPay QR. To: Design a controlled migration model that preserves provider onboarding, QR mapping, transaction feed, settlement feed, refund workflow, health-plan usage, communication ownership, support resolution, and logistics continuity.

6. Business and Product Objectives

Objective	Why It Mattered
Adopt BajajPay across BFHL providers	Create alignment with BajajPay payment infrastructure while supporting BFHL provider journeys.
Onboard new BFHL merchants on BajajPay	Hospitals, doctors, and labs needed a scalable onboarding path.
Migrate existing HealthPay providers	Existing HealthPay QR business needed to move to BajajPay QR without losing historical records.
Keep HealthPay and BajajPay systems in sync	Transaction, settlement, QR, refund, and provider records needed to remain consistent across systems.
Preserve customer and merchant communication	Communication ownership varied by transaction origin; merchant communication remained with BFHL.
Enable health-plan consumption in 3-in-1 journeys	Health-plan benefits needed to be usable beyond BFHL-only payment surfaces.
Define joint support operating model	Customer and merchant issues needed clear BFHL/BFL ownership and escalation paths.

7. Stakeholders and Systems

Stakeholder / System	Role in the Transition
BFHL / HealthPay	Owns provider relationship, merchant communication, BFHL-side systems, HealthPay data, and partner-facing visibility.
BFL / BajajPay	Owns BajajPay QR infrastructure, payment rail capabilities, settlement APIs/feeds, refund APIs/webhooks, and dashboard.
Doctors / Labs / Hospitals	Merchants/providers to be onboarded or migrated from HealthPay QR to BajajPay QR.
FOS / Field Teams	Responsible for onboarding, QR placement, consent, logistics execution, and provider migration actions.
Customers	Pay via HRx/web, 3-in-1 app, or other UPI apps; communication depends on transaction origin.
SFDC / S1 / DRx / Provider Portal	Operational systems for onboarding, provider mapping, PMS/merchant visibility, support tickets, and transaction views.
Support Teams	Customer and merchant query resolution across BFHL/BFL depending on payment origin.

8. Problem Statements

Core Problem Statement

BFHL and BFL needed a controlled way to transition HealthPay providers to BajajPay QR infrastructure because onboarding, QR generation, QR mapping, transaction feeds, settlement feeds, refunds, health-plan consumption, merchant communication, customer support, logistics, and provider migration were distributed across multiple systems and teams, creating high operational complexity, failure-tracking risk, and rollout dependency.

User / Team	Problem Statement
Providers / Merchants	Providers needed a smooth migration to BajajPay QR without losing payment visibility, settlement visibility, communication continuity, or operational support.
Field / FOS Teams	Field teams needed structured onboarding, QR mapping,

	consent, logistics, and QC flows because the transition involved both fresh onboarding and old QR migration.
Finance / Settlement Teams	Finance teams needed transaction and settlement feed continuity because QR migration could otherwise disrupt settlement reconciliation and payout communication.
Customer Support	Support teams needed clear origin-based case ownership because customers could pay through HRx, 3-in-1, or other UPI apps.
Merchant Support	Merchant support teams needed BFHL-first support with BFL L2 support for BajajPay-origin cases.
Product / Technology Teams	Product and technology teams needed API/webhook contracts, failure handling, retry logic, QR status sync, and settlement/refund data exchange across BFHL and BFL.

9. As-Is Complexity and Risk Diagnosis

The merger decision document explicitly highlighted the risk of too many variable components, multiple stakeholders, multiple systems across group companies, difficult failure tracking, and increased TAT across technology, product, operations, logistics, and support. This risk framing is important because the project required operational design, not only API development.

Complexity Area	Risk / Product Diagnosis
Multiple provider types	Doctors, labs, and hospitals had different onboarding and QR/logistics requirements.
Existing and fresh merchants	Migration and onboarding could not use the exact same process. Existing HealthPay UPI IDs needed mapping to new BajajPay UPI IDs.
Multiple transaction origins	HRx/web, 3-in-1, and other UPI apps created different communication and support responsibilities.
Health-plan usage	Health-plan benefits required product API and 3-in-1 visibility; not all UPI journeys could support plan utilization.
Settlement ownership	Settlement feeds and communication needed synchronization between HealthPay and BajajPay.
Refund and chargeback handling	Refund API/webhook and chargeback proof-of-delivery flows needed cross-company coordination.
QR logistics	QR template, printing, delivery, standee reuse, and QC created rollout dependencies beyond software.

10. Solution Tracks

Solution Track	Capability Scope
Merchant Onboarding	Doctor, lab, hospital onboarding; bulk upload; document/data capture; consent; QC; BFL QR enablement.
Existing Provider Migration	Share existing provider list, common parameters, old HealthPay UPI IDs; receive BajajPay UPI IDs; map new IDs against old IDs; preserve historical HealthPay transaction records.
QR Processes	QR generation, QR template design, QR logistics, QR scanning, multi-QR support, QR generation API, get QR API, edit/update QR API.
Transaction Feeds	Real-time transaction status API/webhook from BFL to BFHL,

	success/failure handling, retry logic, mandatory transaction metadata.
Settlements	Settlement API and SFTP settlement feed, date/QR-based settlement detail pull, synchronized settlement communication.
Refunds and Chargebacks	Refund API/webhook, partial/full refund, refund status tracking, chargeback notification and proof-of-delivery process.
Merchant Capabilities	QR visibility, transaction feed, settlement feed, cancellation/refund process, financial/communication update tickets, QR request enablement.
Help and Support	Customer support and merchant support case creation, case routing, BFHL/BFL ownership split, status feeds to BFHL SFDC for analytics.

11. Product Requirements and Integration Design

11.1 Merchant Onboarding and Migration

- BFHL collects provider details and documents through existing systems and field workflows.
- BFHL shares provider data and QC status with BFL/BajajPay systems.
- BFL generates QR codes and shares QR/UPI details back to BFHL.
- Existing HealthPay UPI IDs are mapped against new BajajPay UPI IDs.
- Historical HealthPay QR transaction records are not removed after migration.
- Bulk onboarding and bulk QR generation are required for labs, hospitals, and doctors.

11.2 QR and Transaction Integration

- QR activation requires provider identifiers such as doctor/lab/hospital name, location, or custom field.
- Transaction responses from BFL to BFHL are expected through webhook integration.
- Webhook responses should cover success/failure states and avoid pushing pending states until they move to success or failure.
- BFHL is expected to return HTTP 200 to acknowledge response capture, with retry and alert logic in failure scenarios.
- BFL provides transaction status API for fetching status by transaction ID.

11.3 Settlement, Refund, and Support Integration

- Settlement API and SFTP settlement feeds provide settlement details to BFHL.
- Settlement communication timing and text need synchronization across HealthPay and BajajPay.
- Refund API/webhook supports refund initiation and real-time refund status updates.
- Partial and full refunds are required.
- Customer support ownership depends on transaction origin: HRx handled by BFHL; 3-in-1 handled by BFL with BFHL analytics feed.
- Merchant support is BFHL first-level support, with BFL as L2 for relevant payment-origin cases.

12. GTM and Migration Planning

The source note describes a phased GTM and migration plan. CUG was planned with 5 fresh doctors and 5 existing doctors. Post-CUG, 99% of existing HealthPay doctors business was planned for migration to BajajPay QR codes by 31 Mar 2024. Remaining non-transacting HealthPay doctors were planned for migration by 30 Jun 2024 for compliance. Labs and hospitals were to continue on HealthPay until the doctor migration POC, with later migration planning.

Migration Segment	Source-Backed Plan
CUG	5 fresh doctors and 5 existing doctors.

Existing HealthPay doctors	Planned migration of 99% of existing HealthPay doctor business to BajajPay QR by 31 Mar 2024.
Remaining non-transacting doctors	Planned migration by 30 Jun 2024 for compliance.
Labs and hospitals	46 labs/hospitals to continue on HealthPay initially and move after doctor migration POC.
Final rollout caveat	Samadhan moved to another project during final stages; final rollout should not be claimed as personally owned.

13. Metrics and Success Indicators

The source note lists planned/target metric progression across Nov 2023 to Mar 2024. These are useful as business ambition and GTM tracking indicators, but they should not be presented as personally achieved final outcomes unless later validated after rollout.

Metric	Nov 2023 Actual	Mar 2024 Target / Projection
Total transactions	49,833	85,000
Cashless transactions	80	445
Total GMV	Rs. 6.38 Cr	Rs. 10.2 Cr
Cashless GMV	Rs. 44,755	Rs. 2,83,338
Payment success %	80%	95%
T+1 settlement %	99.87%	99.95%
Unique transacting customers	40,095	50,000
Unique transacting doctors	1,570	2,983
Unique transacting providers	36	66
Scan to Pay %	15.36%	40%

Metric Caveat

These numbers should be positioned as planned/target tracking metrics from the project note, not as final achieved impact owned by Samadhan, because he moved to another project during final stages.

14. Risks, Challenges, and Controls

Risk / Challenge	Control / Design Response
FOS onboarding dependency on S1	Track FOS onboarding as prerequisite to CUG/GTM.
Reusability of 6,000 old HealthPay QR standees	QR logistics and standee reuse planning.
Delayed Off-Us Settlement UTR update	Settlement feed/API/SFTP design and delayed-status tracking.
PayU double payment refund tracking	Refund API/webhook and support case workflow.
Failover if PayRx does not capture BajajPay transaction details	Failover mechanism identification and alerting requirement.
Too many variable components	Structured solution tracks, ownership split, and UAT before rollout.
Joint BFHL/BFL support complexity	Define L1/L2 support ownership and case routing by origin.

15. Design Thinking Process

Stage	Application in HealthPay-BajajPay Transition
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Empathize	Understood provider, field, finance, support, customer, and internal stakeholder impact of QR migration. The pain was not only technical; it affected onboarding, logistics, support, settlements, refunds, and communication.
Define	Reframed the problem from QR replacement to controlled cross-company payment infrastructure migration with continuity of provider operations and transaction visibility.
Ideate	Broke the project into onboarding, QR, transaction, settlement, refund, support, merchant capability, logistics, and health-plan consumption tracks.
Prototype / Specify	Defined requirement document, API/webhook expectations, QR/logistics requirements, merchant support flows, and transition tracker items.
Test / Validate	Supported UAT and integrated testing; source note confirms UAT testing was successful and code moved to production from both ends.
Transition	Final CUG/GTM rollout continued with remaining dependencies after Samadhan moved to another project.

16. Your Role and Contribution

Samadhan worked on the HealthPay-BajajPay merger in parallel to Partner Center. His role should be positioned around product requirement definition, solutioning, integration coordination, QR migration planning, SOP/logistics/service design, and UAT support. Because he moved to another project in the final stages, final rollout ownership should be avoided unless separately confirmed.

Contribution Area	Specific Contribution
Requirement Definition	Created/revised requirement document and defined product requirements across onboarding, QR, transactions, settlements, refunds, support, and merchant capabilities.
Solutioning	Contributed to approach finalization, scope definition, scope closure, and requirement finalization.
Merchant Migration Planning	Worked on existing HealthPay-to-BajajPay provider migration logic and UPI/QR mapping requirements.
SOP and Operations Design	Contributed to logistics and service frameworks, service SOP, logistic SOP, QR template design, and QR order processing.
Integration Coordination	Supported coordination across BFHL/eBH and BFL teams for transaction feeds, settlement data, QR mapping, and onboarding dependencies.
UAT Support	Participated in business UAT and CUG preparation before transition to another project.

17. Impact Positioning

The strongest evidence-backed impact statement is: The integration between BajajPay and HealthPay was completed, UAT was successful, and the code base was moved to production from both ends. The initiative created the foundation for HealthPay provider migration to BajajPay QR codes, with planned migration across fresh and existing doctors and later labs/hospitals.

The honest caveat is equally important: Samadhan was moved to another project during the final stages, so the document should position his contribution as solutioning, requirement ownership, UAT support, and transition-stage work - not as full final commercial rollout ownership.

Do Say	Avoid Saying
Worked on requirement definition, solutioning, migration planning, SOP/logistics/service design, and UAT support.	Owned final rollout end-to-end until market-wide migration.
Integration completed, UAT successful, and code moved to production from both ends as per project note.	I personally achieved all later GTM targets.
Defined cross-system requirements for QR migration, transaction feeds, settlement feeds, refunds, support, and health-plan consumption.	It was just a QR replacement project.

18. Interview / Portfolio Narrative

In parallel to Partner Center, I worked on the HealthPay-BajajPay QR transition initiative. The objective was to migrate BFHL HealthPay provider QR infrastructure to BajajPay while keeping provider onboarding, QR generation, transaction feeds, settlement feeds, refund handling, health-plan consumption, support workflows, and communication ownership intact across BFHL and BFL systems.

The key product insight was that QR migration was not just a payment rail switch. It affected doctors, labs, hospitals, field teams, finance, support teams, customers, merchant communication, logistics, and partner-facing transaction visibility. If any part of the migration was poorly designed, it could break settlement continuity, refund tracking, support ownership, or provider trust.

My contribution focused on requirement definition, solutioning, existing HealthPay-to-BajajPay migration planning, QR generation and mapping requirements, transaction and settlement feed requirements, refund workflows, QR logistics/service SOP inputs, merchant support workflows, and UAT support. The integration reached successful UAT and production movement from both sides. I was moved to another project during the final stages, so final CUG and broader GTM rollout ownership transitioned to other stakeholders.

19. Resume Bullet and One-Line Framing

Resume Bullet

Contributed to the HealthPay-BajajPay QR transition initiative, defining requirements for provider onboarding, HealthPay-to-BajajPay QR migration, transaction/settlement feeds, refund workflows, QR logistics, merchant support, and health-plan consumption across HRx/3-in-1 journeys; supported solutioning and UAT until production movement before transitioning to another project during final rollout.

One-Line Framing

Worked on the HealthPay-BajajPay QR transition as a parallel HealthPay initiative, helping design the cross-company migration model for provider onboarding, QR infrastructure, transaction/settlement continuity, refund handling, support ownership, and health-plan consumption before moving to another project in the final rollout stage.

Source basis: BajajPay HealthPay Note, BFHL requirement document v3, merger decision deck, and user-confirmed role/transition context.